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TYPES OF CANCER

Head and neck cancers



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What are head and neck cancers?

Head and neck cancer is a term that describes a range of cancers that begin in the head and neck region. Cancer can occur inside the sinuses, nose, [mouth](#) and salivary glands down through the [throat](#). Although these cancers are different, they are treated similarly, so are considered as a group.

Symptoms

Diagnosis

Treatment

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Understood



Skin cancers that start in the head and neck area have recently been classified as a type of head and neck cancer, whereas cancers that start in the eye, brain, oesophagus, thyroid and trachea, or in bone or muscle of the head and neck area are not usually classified as head and neck cancers.

It is estimated that 5531 people were diagnosed with a head and neck cancer in 2024 (excluding skin cancers).

► **PREVENTION**

► **PROGNOSIS**

Signs and symptoms of head and neck cancer

Symptoms depend on the site of the cancer, however you may have one or more of the following symptoms:

pain

swelling

a hoarse voice

a sore throat that doesn't get better

coughing all the time

difficulties swallowing

bad breath

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Causes of head and neck cancers

There are a number of risk factors associated with the development of head and neck cancers. The main risk factors are alcohol consumption, smoking tobacco (cigarettes, cigars and pipes) and having had [human papillomavirus](#) (HPV). If you drink and smoke, your risk is much greater than if you only drink or only smoke.

Head and neck cancers are more common in people over the age of 40. Men are about three times more likely to develop a head or neck cancer than women. Aboriginal and Torres Strait Islander people are more likely to develop head and neck cancer than other Australians.

Other risk factors include:

- having persistent sores or red or white patches in the mouth

- breathing in asbestos fibres, wood dust or certain chemicals

- viruses including the Epstein-Barr virus have been linked to head and neck cancers

- a weakened immune system

- oral hygiene - poor oral health including gum disease have been linked to cancers of the oral cavity

- being overweight or obese

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having radiation therapy to the head and neck area in the past

inheriting a condition linked to head and neck cancer such as Fanconi anaemia or Li-Fraumeni syndrome

Diagnosis of head and neck cancers

If you notice any symptoms, contact your GP. You can also tell your dentist about any mouth sores, swelling or change of colour in your mouth as dentists are trained to look for signs of mouth cancer.

Your doctor will physically examine your neck, ears, eyes, nose, mouth or throat, depending on your symptoms. Other tests will use an endoscope, needle or surgery to take a biopsy.

Nasendoscopy

Your doctor will examine your nose and throat using a thin flexible tube with a light and camera on the end called a nasendoscope or flexible laryngoscopy. The nasendoscope will look at your nasal cavity, larynx, nasopharynx and oropharynx. This is done under local anaesthetic.

Micro-laryngoscopy

Your doctor will look at your voice box and throat using a tube with a light and camera on the end (laryngoscope) and take a biopsy (tissue sample). This is done in

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Your doctor will remove tissue samples or cells from the affected area and these will be examined under a microscope by a pathologist to check for any cancer cells.

X-rays

You may have x-rays to check for tumours or damage to the bones.

CT scan

A computerised tomography (CT) scan uses x-ray beams to create detailed cross-section images of the inside of your body.

MRI

A magnetic resonance imaging (MRI) scan uses powerful magnet and radio waves to create detailed cross-section images of inside your body. This test usually takes between 30 and 90 minutes.

PET-CT scan

A positron emission tomography (PET) scan combined with a CT scan helps pinpoint the location of any abnormalities. It is recommended to help diagnose laryngeal, pharyngeal or oral cancers or to see if cancer has spread.

Ultrasound

You may have an ultrasound to look at the salivary glands, the thyroid and lymph glands in the neck.

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After being diagnosed with a head or neck cancer, you may feel shocked, upset, anxious or confused. These are normal responses. A diagnosis of a head or neck cancer affects each person differently. For most it will be a difficult time, however some people manage to continue with their normal daily activities.

You may find it helpful to talk about your treatment options with your doctors, family and friends. Ask questions and seek as much information as you feel you need. It is up to you as to how involved you want to be in making decisions about your treatment.

Find out more about the best head and neck cancers care:

What should happen next?

This resource can help guide you and your loved ones after your diagnosis.

**GUIDE TO BEST CARE FOR HEAD AND NECK
CANCERS**



Treatment of head and neck cancers

Treatment for head and neck cancers can affect your teeth, gums and mouth. Some preparations before treatment begins may include:

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consulting a dietitian to help improve your diet and nutrition as you may lose a lot of weight

beginning an exercise program to help build up your strength

quitting smoking as you may not respond to treatment as well as someone who doesn't smoke

avoiding alcohol as it can irritate mouth sores from the cancer or the treatment.

Surgery

Surgery is used to remove the cancerous tissue while preserving as much of the functions of the head and neck area as possible including breathing, swallowing and talking. The type of surgery you have will depend on the type of head and neck cancer you have.

Radiation therapy (radiotherapy)

Radiation therapy uses a controlled dose of radiation to damage or kill cancer cells. It is sometimes used on its own as the main treatment for head and neck cancers

Radiation therapy can be used after surgery (adjuvant treatment) and is sometimes used with chemotherapy.

Chemotherapy

Chemotherapy uses specific drugs to kill cancer cells or slow their growth. The drugs

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Targeted therapy drugs affect specific features of cancer cells to stop their growth.

Immunotherapy

Immunotherapy, a type of treatment that uses your body's own immune system to fight cancer, may be used to treat head and neck cancers.

Palliative care

In some cases of head and neck cancer, your medical team may talk to you about **palliative care**. Palliative care aims to improve your quality of life by alleviating symptoms of cancer.

As well as slowing the spread of head and neck cancer, palliative treatment can relieve pain and help manage other symptoms. Treatment may include radiotherapy, chemotherapy or other drug therapies.

Treatment Team

Depending on your treatment, your treatment team may consist of a number of different health professionals, such as:

GP (General Practitioner) - looks after your general health and works with your specialists to coordinate treatment.

Ear, nose and throat (ENT) specialist - treats disorders of the ear, nose and throat

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Reconstructive (plastic) surgeon - performs any complex reconstructive surgery that restores or repairs the body's appearance and function.

Medical oncologist - prescribes and coordinates the course of chemotherapy.

Radiation oncologist - prescribes and coordinates radiation therapy treatment.

Dentist - evaluates and treats the mouth and teeth

Oncology nurse - cares for people who are diagnosed with cancer throughout treatment.

Aboriginal and Torres Strait Islander liaison officer - if you identify as Aboriginal or Torres Strait Islander, the liaison officer will support you and your family during treatment and recovery.

Other allied health professionals - such as social workers, pharmacists, and counsellors

Screening for head and neck cancers

There is currently no national screening program for head and neck cancer available in Australia.

White plaques or patches in the mouth (leukoplakia) may precede the development of the cancer.

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The risk of head and neck cancers can be reduced by not smoking (or quitting) and reducing alcohol consumption.

Prognosis for head and neck cancers

Prognosis means the expected outcome of a disease. Your prognosis depends your test results, the type of head and neck cancer, the tumour's HPV status, the rate and depth of tumour growth and other factors such as your age and general health at the time of diagnosis.

The earlier a head and neck cancer is diagnosed the better the outcomes.

Sources

Understanding Head and Neck Cancers, Cancer Council Australia, © 2024. Last medical review of source booklet: February 2024. We thank the reviewers of this booklet: A/Prof Martin Batstone, Oral and Maxillofacial Surgeon and Director of the Maxillofacial Unit, Royal Brisbane and Women's Hospital, QLD; Polly Baldwin, 13 11 20 Consultant, Cancer Council SA; Martin Boyle, Consumer; Dr Teresa Brown, Assistant Director Dietetics, Royal Brisbane and Women's Hospital, Honorary Associate Professor, University of Queensland, QLD; Dr Havlev Dixon. Head. Clinical Support Dentistry Department. WSLHD Oral Health

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